



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سلوي عزت محمود مجدي
المحافظة : القاهرة
الحى / المدينة : السيده زينب
العنوان : السيده زينب
رقم البطاقة : 25408150102921 رقم التليفون : 01116492015
العمر (تاريخ الميلاد) : 15-08-1954 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سلوي عزت محمود مجدي National ID: 25408150102921	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سملوى
عزت محمود مجدى
اش. يعقوب - لا ظو غنى
السيدة زينب - القاهرة

١٩٥٤/٠٣/١٥ ٢٥٤.٠٨.١٥.٠١.٢٩٢١

KL6123995

٢٠٢٤/٠٣ ٢٥٤.٠٨.١٥.٠١.٢٩٢١

ربة منزل

انشى
مسلمة
متزوجة
احمد ابراهيم فهمى البيلوى
البطاقة سارية حتى ٢٠٣١/٠٣/١٥

١



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : مملوكي عذرا محمود مجدي

Sex:

MDT form

3- Medical number:

Female

83197392

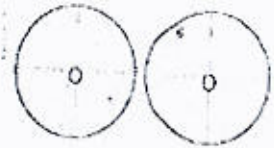
Old Med:

Date:

1005-2024

2-ID number:

Patient Age : 70 Y



Family history

Family history
NVE

Phone

Phone

01116492015

BMI

GAT

REASON OF MDT

REASON OF MDT
POST INCLUSIVE

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity
freeMenopausal
statusMenopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

24/10/2024

Clinical data

Breast Imaging :

Mets Workup :
bone scan:free

BX IDC BX COMMENT	ER S/8 ER COMMENT	PR S/8 PR COMMENT	Her 2 Neu 1 Her 2 Neu COMMENT	SISH SISH COMMENT	Ki67 20
T 4b	N 3c	M 0			

Sonomammography

retroareolar mass 5 x4 cm (18 mm from nipple with microcalcification)

cesni: retroareolar non mass enhancement 110 x90x85mm+multiple pathological axillary LN

PATIENT HISTORY

LT LABC , NACT , AC -->taxol with good clinical response

LT MRM :residual viable IDC GII

free deep margins

infiltrated upper dermis

LN 9/12 WITH ECE YPT3(11CM) N2

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

1- ADJUVANT AI(CONSIDER EXTENDED ADJUVANT)

2- PORT

3- ZOMETA/6 MONTHS

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY Date: 10/10/24	RADIOLOGY	RADIOLOGY Date: 10/10/24
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

الأستاذة / ماري عزت محمود مجدى

5408150102921

657/24

Female

19 Y

Request No. : 13004071

Request Date : 28/09/2024

Verification Date : 13/10/2024

Financial Category : مريض - Out Patient

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

t breast cancer, post NACT.
Left modified radical mastectomy specimen.

Acc.No.	13004071
P.No.	25453150102921
Reg.	28-09-24 (10.57)
Res.	13-10-2024

MRM measured 23x19x6 cm with covering skin ellipse measured 23x17

greyish white firm tumor bed site.

Nipple & areola were unremarkable grossly.

margins: 3 cm away from deep margin.

umber and size: Fibrofatty tissue peice measured 10x8x2 cm entangling 15

mination:

Residual viable invasive duct carcinoma.

tilation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

na in situ: Commedo and solid (30%).

nt: Infiltrated upper dermis without skin ulceration.

volvement: Positive in (9/12) with extracapsular extension.

ins: Free deep margin.

onse to therapy: Partial pathological response.

ologic findings: Effect of therapy in the form of fibrosis and hyalinosis.

r post NACT, MRM specimen:

vasive duct carcinoma, Grade II.

Nabil

24 08:25:48

Page 1 of 2

Printing By : marwa mohamed youss



Name: Mrs. Salwa Ezzat Mahmoud Magdy

Date: 11/3/2024

ID:2224009071

Acc.:4898123

Referring Prof. Dr. Omayya Abdel-Hameed Nassar

Digital 3D soft tissue mammography & tomosynthesis with ultrasound examination of both breasts revealed:

Indication: Left palpable breast lump

- (ACR, b).
- The left breast shows irregular ill-defined spiculated dense mass lesion at the retroareolar region causing nipple retraction and mild overlying skin thickening measures about 4.8 cm. It shows linear segmental pleomorphic microcalcifications within and extending into the inner central region (medial to the index lesion). It measures collectively about 8.0 x 4.5x 5.8 cm at its maximum diameters and about 18 mm from the nipple.
- Normal skin thickness and contour of right side.
- Normal areola / nipple complexes on right side.

Ultrasonographic examination showed:

- The left breast shows ill-defined irregular retroareolar hypoechoic mass lesion measures about 5.1 x 4.0 cm and about 18 mm from the nipple. It shows internal microcalcifications within with medial extension located at 9 ° clock position measures about 4.0 cm long.
- Normal right skin thickness and contour.
- Normal right nipple/areola complex.
- Right non-specific axillary lymph nodes.
- **Left enlarged axillary lymph nodes (>5) with effaced hila at level I, II & III, largest one located at level I measures about 12.6 x 7.2 mm**
- **Left enlarged supraclavicular (>5) lymph nodes with effaced hila, largest one measures about 26.5 x 11.2 mm.**

Opinion:

- **Left retroareolar malignant ill-defined mass lesion with ipsilateral pathological axillary and supraclavicular lymph nodes as described (BIRADS 5)**
- **Right normal study (BIRADS 2)**

Recommendation:

- Tru cut biopsy from the left retroareolar malignant mass
- Dynamic MRI breast study

Much obliged,
Dr. Hedaya Nour, Msc

Typed MA 11/03/24 n.nour

Name: Mrs. Salwa Ezzat Mahmoud Magdy

BI-RADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BI-RADS 0: Incomplete - need additional imaging evaluation or comparison with previous films.
- BI-RADS 1: Normal examination - nothing to comment on - Routine mammography screening is recommended.
- BI-RADS 2: Benign findings - routine mammography screening is recommended.
- BI-RADS 3: Probably benign <2% malignancy - short interval follow-up is suggested.
- BI-RADS 4: Suspicious >2 but <95% - Biopsy should be considered.
 - 4A: low suspicion of malignancy: 2-20%.
 - 4B: Mod suspicion of malignancy: 10-50%.
 - 4C: High suspicion of malignancy: 50-90%.
- BI-RADS 5: Highly suspicious of malignancy >95% - appropriate action should be taken.
- BI-RADS 6: Known - biopsy proven malignancy. Surgical excision when clinically appropriate.

American college of radiology (ACR) scoring of breast density:

- ACR a: almost entirely fatty.
- ACR b: scattered areas of fibro-glandular density.
- ACR c: heterogeneously dense.
- ACR d: extremely dense.

Medical No. : 10068

Patient's Name: Salwa Ezzat Mahmoud Magdy

Date: 2026-09-29

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Examination:

■ **Computed tomography of the neck, chest, abdomen and pelvis**

Technical data:

- 5 mm CT scans of the neck, chest, abdomen and pelvis with IV contrast

Clinical data: known left breast cancer

Comparison: none

Neck findings:

- Normal CT features of the naso-, oro- and hypo-pharyngeal compartments with no evidence of focal mass lesions or areas of abnormal attenuations.
- Normal size and tissue texture of the parotid and submandibular glands with no focal lesions.
- Normal appearance of the laryngeal compartments with no mass lesions or abnormal tissue attenuations. Intact laryngeal skeleton.
- Normal CT appearance of the thyroid gland
- Normal visceral and muscular compartments of the neck with no evidence of mass lesions or suspicious cervical lymphadenopathy.

Chest findings:

- Left mastectomy with clear operative bed
- Clear both lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.
- **Abdomen and pelvis findings:**
- The liver shows enlarged size with smooth contours, diffuse homogeneous low hepatic parenchyma density (enlarged fatty) and no evidence of focal lesions. Right lobe small calcified focus is seen.

Medical No.: 10068

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- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Right renal 7 cm cortical simple cyst, otherwise both kidneys are of normal site, size and shape, no stones, back pressure or masses bilaterally.
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection

Opinion:

- **No definite pulmonary nodules or masses**
- **No suspicious abdominal or pelvis metastatic lesions on CT base**
- **No suspicious bony lesions on CT base.**

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Medical No. : 10068

Patient's Name: Salwa Ezzat Mahmoud Magdy

Date: 2026-09-29

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post left MRM routine follow up.

Findings:

Right breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/hipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections.
- Non specific axilla 12x4mm with persevered fatty hilum . No pathological supra or infra-clavicular lymph nodes.

Conclusion:-

- *Right breast (BIRADS 1): normal study.*
- *Clear left mastectomy bed and axilla.*

Recommendation:-

- *For annual follow up.*

Best Regards,
DR. Marwa M.Hassan

Dr Amal Hegazy